

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase III, Multicenter, Randomized, Open-Label Study Comparing the Efficacy and Safety of Glofitamab (RO7082859) in Combination With Polatuzumab Vedotin Plus Rituximab, Cyclophosphamide, Doxorubicin, and Prednisone (Pola-R-CHP) Versus Pola-R-CHP in Previously Untreated Patients With Large B-Cell Lymphoma
Short Title:	An Open-Label Study Comparing Glofitamab and Polatuzumab Vedotin + Rituximab, Cyclophosphamide, Doxorubicin, and Prednisone Versus Pola-R-CHP in Previously Untreated Patients With Large B-Cell Lymphoma
Protocol Number:	310350774548791924
NCT Number:	NCT06047080
Sponsor Name:	Roche
Investigational Product:	cyclophosphamide
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Progression-free survival (PFS) as determined by Independent Review Facility (IRF)
Secondary Obj.:	1. PFS as determined by the investigator 2. PFS as determined by the investigator and IRF for participants with international prognostic index (IPI) 3-5 3. Event-free survival efficacy causes (EFSeff)

2.2 Background & Rationale

The purpose of this study is to compare the efficacy and safety of glofitamab in combination with polatuzumab vedotin plus rituximab, cyclophosphamide, doxorubicin, and prednisone (Pola-R-CHP) vs Pola-R-CHP in participants with previously untreated CD20-positive large B-cell lymphoma (LBCL).

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING
Target N:	1130

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Previously untreated participants with CD20-positive LBCL
- * Ability to provide tumor tissue
- * International prognostic index (IPI) score 2-5
- * Eastern cooperative oncology group (ECOG) performance status of 0, 1, or 2

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- * At least one bi-dimensionally measurable lesion, defined as ≥ 1.5 cm in its longest dimension as measured by CT or MRI
- * Left ventricular ejection fraction (LVEF) $\geq 50\%$ on cardiac multiple-gated acquisition (MUGA) scan or cardiac echocardiogram (ECHO)
- * Adequate hematologic function
- * Negative HIV test at screening with exceptions as defined by the protocol
- * Negative SARS-CoV-2 antigen or PCR test

4.2 Exclusion Criteria

- * Contraindication to any of the individual components of Pola-R-CHP or glofitamab, including prior receipt of anthracyclines, or history of severe allergic or anaphylactic reactions to humanized or murine monoclonal antibodies, or known sensitivity or allergy to murine products
- * Prior solid organ transplantation
- * Participants receiving systemic immunosuppressive agent such as, but not limited to cyclosporin, azathioprine, methotrexate, thalidomide, and anti-tumor necrosis factor agents within 4 weeks prior to first dose of study treatment
- * Current Grade ≥ 1 peripheral neuropathy by clinical examination or demyelinating form of Charcot-Marie-Tooth disease
- * History of indolent lymphoma (e.g., Follicular Lymphoma, Marginal Zone Lymphoma, Waldenstrom macroglobulinemia)
- * Current diagnosis of the following: Follicular lymphoma grade 3B; transformations of indolent B-cell lymphomas (e.g., de novo transformed follicular lymphoma); mediastinal grey zone lymphoma; primary mediastinal (thymic) large B-cell lymphoma; Burkitt lymphoma; primary large B-cell lymphoma of immune-privileged sites (encompassing primary diffuse large B-cell lymphoma of the CNS, primary large B-cell lymphoma of the vitreoretina and primary large B-cell lymphoma of the testis); primary effusion DLBCL; and primary cutaneous DLBCL, leg type
- * Primary or secondary CNS lymphoma at the time of recruitment or history of CNS lymphoma
- * Prior treatment with systemic immunotherapeutic agents
- * Prior use of any monoclonal antibody for the purposes of treating cancer within 3 months of the start of Cycle 1
- * Any investigational therapy for the purposes of treating cancer within 28 days prior to the start of Cycle 1
- * Prior radiotherapy to the mediastinal/pericardial region
- * Prior therapy for LBCL, with the exception of corticosteroids
- * Corticosteroid use ≥ 30 mg/day of prednisone or equivalent, for purposes other than lymphoma symptom control
- * History of other malignant or non-malignant diseases that could affect compliance with the protocol or interpretation of results
- * Significant or extensive history of cardiovascular disease
- * Recent major surgery (within 4 weeks prior to the start of Cycle 1), other than for diagnosis
- * Current or past history of central nervous system (CNS) disease, such as stroke, epilepsy, CNS vasculitis, or neurodegenerative disease
- * Known or suspected chronic active Epstein-Barr viral infection
- * Known or suspected history of hemophagocytic lymphohistiocytosis (HLH)
- * Active autoimmune disease requiring treatment
- * Clinically significant liver disease
- * Live, attenuated vaccine within 4 weeks before study treatment infusion on Day 1 of Cycle 1 or anticipation that such a

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live, attenuated vaccine will be required during the study. Live vaccines during the study and until participants B cells recover are prohibited

- * Any active infection within 7 days prior to Cycle 1 Day 1 that would impact participant safety
- * Suspected active or latent tuberculosis
- * Positive test results for chronic hepatitis B infection, hepatitis C, or the human T-lymphotropic virus type 1 (HTLV-1)
- * History of progressive multifocal leukoencephalopathy

11. Study Identifiers & Governance

Primary ID: 310350774548791924

Registry Number: NCT06047080

Sponsor: Roche

Study Title: An Open-Label Study Comparing Glofitamab and Polatuzumab Vedotin + Rituximab, Cyclophosphamide, Doxorubicin, and Prednisone Versus Pola-R-CHP in Previously Untreated Patients With Large B-Cell Lymphoma

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____